

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Atorvastatin 20mg/40mg/80mg tablets for the treatment of Cholesterol/Lipid Management

Summary of NICE / NICE CKS Guidelines for Cholesterol/Lipid Management

NICE Guidelines: Lipid Management (CG181) and CVD Risk

- **CVD Risk Assessment:** Use QRISK3 calculator to assess 10-year CVD risk. Patients with QRISK3 $\geq 10\%$ are candidates for statin therapy.
- **Primary Prevention:** Offer atorvastatin 20mg daily to adults with QRISK3 $\geq 10\%$ and no prior CVD. Target is $\geq 40\%$ reduction in non-HDL cholesterol.
- **Secondary Prevention:** Offer atorvastatin 80mg daily to patients with established CVD (acute coronary syndrome, prior MI, stable angina, stroke, TIA, peripheral arterial disease).
- **Lipid Monitoring:** Check fasting lipid profile at baseline, 4-12 weeks after initiation/dose change, then annually. Monitor LDL-C and non-HDL-C targets.
- **Lifestyle:** Counsel on diet, weight loss, exercise, smoking cessation, and alcohol reduction as part of CVD risk reduction.

Patient Group Direction

for the supply/administration of

Atorvastatin 20mg/40mg/80mg tablets

for the treatment of

Cholesterol/Lipid Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own

	employer All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	- All users must be up to date with CPD requirements and appraisal - All users must be up to date with MHRA and safety alerts with regards to medical products - All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Primary prevention of cardiovascular disease in patients with QRISK3 $\geq 10\%$, or secondary prevention in patients with established CVD
Inclusion criteria	Aged 18 years and over Primary prevention: QRISK3 $\geq 10\%$ calculated within previous 5 years Secondary prevention: Established cardiovascular disease (prior MI, acute coronary syndrome, stable angina, stroke/TIA, peripheral arterial disease) Able to take oral medication
Exclusion criteria	Active liver disease or ALT/AST >3 times upper limit of normal Unexplained persistent elevations of liver transaminases Pregnancy or breastfeeding Hypersensitivity to atorvastatin or other statins Concurrent ciclosporin therapy Severe renal impairment with drug interaction risk
Cautions	Hepatic impairment: use with caution; reduce dose if moderate impairment; avoid if severe History of liver disease or high alcohol intake (>14 units/week in women, >21 in men) Renal impairment: eGFR <30 mL/min/1.73m² may require dose adjustment Hypothyroidism: may increase statin effect; monitor TSH and lipids CYP3A4 inhibitor interactions (macrolides, azoles, protease inhibitors, some calcium channel blockers) Risk factors for myopathy: elderly, female, low body weight, multiple medications, muscle symptoms Diabetes mellitus (monitor glucose control)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Atorvastatin 20mg, 40mg, and 80mg tablets
Legal category	POM
Storage	Store below 25°C in a dry place, protected from light
Route/method of administration	Oral, swallowed whole with water
Dose and frequency	Primary prevention: Start 20mg once daily Secondary prevention: Start 80mg once daily Can be taken at any time of day with or without food Do not increase dose without lipid results
Quantity to be administered and/or supplied	28 tablets per month
Maximum or minimum treatment period	Ongoing. Check lipids at 3 months after initiation/dose change, then annually. Target $\geq 40\%$ reduction in non-HDL-C from baseline.
Adverse effects	Common: Nasopharyngitis, arthralgia (joint pain), diarrhoea, myalgia (muscle aches), abdominal pain, constipation, nausea Uncommon: Liver enzyme elevation, insomnia, memory impairment, erectile dysfunction Rare: Rhabdomyolysis (severe muscle breakdown), hepatotoxicity (liver damage), severe myopathy, pancreatitis, peripheral neuropathy

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows:

	- Adults aged 18 years and over - keep records for 8 years - Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with atorvastatin tablets.
Follow-up advice to be given to patient or carer	Take the tablet once daily at the same time, preferably in the evening Do not stop the medication without consulting your doctor Report any unexplained muscle pain, weakness, or tenderness to your doctor Inform your doctor of all other medications, especially if taking azithromycin, fluconazole, or calcium channel blockers Attend blood tests as arranged (fasting lipid profile, liver and kidney function) Maintain lifestyle changes: reduce saturated fats, increase fibre intake, exercise regularly, lose weight if overweight, stop smoking Avoid alcohol excess or discuss alcohol use with your doctor If pregnant or planning pregnancy, inform your doctor immediately Attend all follow-up appointments for lipid monitoring and CVD risk reassessment

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2 - NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources - NICE CKS Guidance: Lipid Management NICE CG181 - British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines
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Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
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Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025